AR-10 Tracheostomy Tube Emergencies

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Tracheostomy or laryngectomy patient with distress, cyanosis, altered mental status, copious secretions, obstruction, displacement, decannulation, or bleeding. First distinguish tracheostomy from total laryngectomy.

Field flow

1. Identify	Ask caregiver and inspect stoma/tube. If uncertain, apply oxygen to both face and stoma.
2. Obstruction	AEMT/Paramedic: remove valve, cap, HME/filter, and inner cannula. Do not remove voice prosthesis.
3. Suction	Use patient-specific catheter/depth when known. Never force resistance.
4. Rescue	If catheter will not pass or tube is displaced: deflate cuff, reassess, remove trach tube if obstruction/displacement is established, and ventilate via available airway.

Provider-level actions

Level	Approved actions
EMT	Use caregiver/equipment, distinguish tracheostomy vs laryngectomy, provide oxygen, assist stoma ventilation, and prepare suction/replacement equipment.
AEMT	All EMT care plus remove simple obstructions, authorized suction, and authorized tracheostomy replacement. ETT through stoma is not an AEMT procedure.
Paramedic	All prior care plus persistent obstruction/displacement management. For a mature tract, patient spare tube or one size smaller; cuffed ETT through stoma is a rescue option when necessary.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• A confirmed total laryngectomy disconnects the upper airway from the lungs: oxygenate and ventilate through the stoma only.

• For a fresh/immature tract, do not blindly replace the tube because of false-passage risk; prioritize oxygenation/ventilation and rapid transport.

• Deflate cuff before removing a cuffed tracheostomy tube.

• Significant tracheostomy bleeding is potentially life-threatening; expedite transport and contact Medical Control early.

Linked pathways

AR-01 Adult Airway; AR-08 Post-Intubation / BIAD Management; AR-09 Ventilator Emergencies.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.